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CITY OF BERKELEY

BERKELEY CITY COUNCIL SPECIAL MEETING

February 24, 2026

WEBVTT

1
00:00:20.540 --> 00:00:22.049
Board Room: I don't need it right now, actually.

2
00:00:34.900 --> 00:00:40.780
Board Room: No, I mean, sometimes they're not powerful enough, but the big fat ones usually Thank you for your support.

3
00:00:42.190 --> 00:00:43.350
Board Room: Gatehouse.

4
00:00:49.870 --> 00:01:06.660
Board Room: Yes, we're ready. All right. Hello, everyone, good evening. I am calling to order a special meeting of the Berkeley City Council. Today is Tuesday, February 24th, 2026. Clerk, could you please take the roll?

5
00:01:07.130 --> 00:01:10.950
Board Room: Okay, Councilmember Kasarwani is absent.

6
00:01:11.440 --> 00:01:13.060
Board Room: Councilmember Taplin?

7
00:01:13.220 --> 00:01:14.530
Board Room: is absent.

8
00:01:15.240 --> 00:01:16.710
Board Room: Councilmember Bartlett?

9
00:01:16.920 --> 00:01:18.030
Board Room: is absent.

10
00:01:18.940 --> 00:01:20.259
Board Room: Councilmember Tragum?

11

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00:01:20.390 --> 00:01:21.170
Board Room: Pleasant.

12
00:01:21.300 --> 00:01:22.200
Board Room: O'Keefe?

13
00:01:22.550 --> 00:01:23.420
Board Room: I'm here.

14
00:01:23.780 --> 00:01:25.390
Board Room: Blackaby, here.

15
00:01:26.370 --> 00:01:27.440
Board Room: Lunapara?

16
00:01:28.240 --> 00:01:28.960
Board Room: Here.

17
00:01:29.450 --> 00:01:34.800
Board Room: Humbert… And Mary Ishii? Here. Okay, quorum is present.

18
00:01:35.270 --> 00:01:46.339
Board Room: Very good, so today we have a very special work session. Only one item on this agenda, it's the Community Health Improvement Plan, so I'm going to pass it over to Director Scott Gilman.

19
00:01:46.500 --> 00:01:48.570
Board Room: For our presentation. Thank you.

20
00:01:48.860 --> 00:01:59.320
Board Room: Thank you, Mayor. Yes, again, to introduce myself, I'm Scott Gilman, I'm the Director of Health, Housing, and Community Services, and I'd like to introduce Tanya Bustamati, who is our Deputy Director.

21
00:01:59.550 --> 00:02:07.459
Board Room: We also have several staff in the room that have helped put this information together over the last year, and if they could just raise their hand, please.

22
00:02:08.560 --> 00:02:14.259
Board Room: We've got our crew there. We have members of the Community Health Commission.

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23
00:02:15.060 --> 00:02:16.280
Board Room: That are present.

24
00:02:16.760 --> 00:02:23.490
Board Room: And then at the end, we'd like to recognize the members of the steering committee that actually work specifically, on this project.

25
00:02:23.860 --> 00:02:25.689
Board Room: So with that, we'll start our slideshow.

26
00:02:27.540 --> 00:02:37.580
Board Room: So the agenda that we have for you tonight is we're going to do an overview of the Community Health Improvement Plan, better known as the CHIP. We have a short video that we'd like to show you.

27
00:02:38.060 --> 00:02:48.369
Board Room: We have an exciting new dashboard that we plan to demonstrate with some data, and then we'll talk about implementation, next steps, and questions and recognition.

28
00:02:49.020 --> 00:02:49.700
Board Room: Tanya?

29
00:02:52.680 --> 00:03:05.040
Board Room: Okay, so the purpose of our Community Health Improvement Plan is really to describe how the Health Department and the community will work together to improve the health of the citizens of Berkeley.

30
00:03:05.370 --> 00:03:12.459
Board Room: The Community Health Improvement Plan, or CHIP, identifies health priorities that came out of our Community Health Assessment.

31
00:03:12.680 --> 00:03:15.120
Board Room: And strategies for how to address them.

32
00:03:15.350 --> 00:03:24.509
Board Room: In addition, the CHIP is required for national public health accreditation, which is something that our department, HHCS, is actively working towards.

33
00:03:24.840 --> 00:03:37.100

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Board Room: It is also a requirement for local health jurisdictions to align their CHIP with behavioral health strategies that the health jurisdiction will be focusing on as part of our Behavioral Health Services Act plan.

34
00:03:43.120 --> 00:03:49.979
Board Room: To guide this work, four guiding principles were used to draft the high-level goals and objectives in developing the CHIP.

35
00:03:50.390 --> 00:03:57.770
Board Room: The first one is to balance ambition with feasibility. So, setting goals that are achievable and realistic.

36
00:03:58.170 --> 00:04:01.400
Board Room: The second one is to cultivate cross-sector work.

37
00:04:01.800 --> 00:04:09.049
Board Room: As you know, at any given time, there are various planning processes, commission meetings, policy debates that are happening.

38
00:04:09.280 --> 00:04:22.060
Board Room: It's critical for us as city staff, to be aware of and collaborate with other city departments and community members on the work that we're all doing so that we're not operating in silos.

39
00:04:22.990 --> 00:04:38.420
Board Room: The third principle is to emphasize prevention. So to really address the root causes and not just the health outcomes. And lastly, establishing a long-term cycle of assessment, improvement planning, and implementation.

40
00:04:38.820 --> 00:04:52.600
Board Room: In fact, local health departments and health plans are required to collaborate on assessment and planning processes every 3 years. So this cycle is an opportunity to build trust and achieve long-term impact.

41
00:04:56.210 --> 00:04:59.400
Board Room: So I'd like to talk a little bit about how we got here.

42
00:04:59.930 --> 00:05:04.000
Board Room: Our approach was comprehensive and community-driven.

43

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00:05:04.160 --> 00:05:17.330
Board Room: The first step, we took 3 main steps, and the first one was to do a landscape scan, where we took an initial snapshot of the health of the Berkeley community.

44
00:05:17.760 --> 00:05:28.539
Board Room: We worked with a consulting firm, JSI, to gather information, activities, and resources through both qualitative and quantitative data synthesis.

45
00:05:29.000 --> 00:05:37.970
Board Room: JSI helped to conduct key informant interviews with local leaders and community members. They conducted a handful of focus groups.

46
00:05:38.120 --> 00:05:48.519
Board Room: And we also partnered with them to develop a community survey that was distributed throughout Berkeley, of which we collected over 320 responses.

47
00:05:49.530 --> 00:05:59.040
Board Room: The qualitative data that we collected focused on the community environment, access to services, lived experiences, etc.

48
00:05:59.520 --> 00:06:08.420
Board Room: During this first phase, a community steering committee of 10 community members with diverse backgrounds and experiences was convened.

49
00:06:08.680 --> 00:06:14.120
Board Room: This committee was instrumental in analyzing the data that was collected.

50
00:06:16.840 --> 00:06:25.670
Board Room: The second step was to develop our community health assessment by taking a thorough look at health, safety, and equity in the city.

51
00:06:25.810 --> 00:06:36.949
Board Room: The health assessment identified priority issues and priority populations, and enabled the community steering committee to distill the information into six key findings.

52
00:06:37.560 --> 00:06:50.789
Board Room: From the community health assessment, our steering committee, in collaboration with city staff, developed the Health Improvement Plan,

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which lays out a realistic response to the priorities that were identified in the health assessment.

53
00:06:57.130 --> 00:07:07.550
Board Room: As part of the community health assessment process, the steering committee identified two broad goals for the CHIP, for both their universality and potential impact.

54
00:07:08.180 --> 00:07:12.489
Board Room: The first goal was to address disparities in life expectancy.

55
00:07:12.810 --> 00:07:17.189
Board Room: And the second goal is to increase community power and partnership.

56
00:07:18.270 --> 00:07:33.479
Board Room: While the health assessment identified several key findings and health topics, it elevated four priority areas under the umbrella of these two broad goals. Those are housing, environmental health, behavioral health, and community safety.

57
00:07:34.000 --> 00:07:40.629
Board Room: Housing is defined as being able to live in a place that is safe, affordable, and stable.

58
00:07:41.140 --> 00:07:52.309
Board Room: Environmental health was defined as being safe from things like pollution and wildfires, and having clean resources and access to green spaces.

59
00:07:53.480 --> 00:07:55.510
Board Room: The behavioral health area.

60
00:07:55.620 --> 00:08:01.920
Board Room: Highlighted that mental health problems are common in our community, and our members need access to care.

61
00:08:02.580 --> 00:08:09.270
Board Room: Community safety highlighted how often people get hurt and experience violence in our community.

62
00:08:10.230 --> 00:08:24.890

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Board Room: The steering committee worked closely with city staff to review the health assessment data, as well as existing Berkeley policies, resources, and initiatives to establish a high-level objective for each priority area, which you see here.

63
00:08:25.770 --> 00:08:33.329
Board Room: Additionally, these high-level priorities, or these high-level objectives, align with strategic priorities.

64
00:08:33.620 --> 00:08:37.059
Board Room: Across other local, county, and state agencies.

65
00:08:38.360 --> 00:08:58.220
Board Room: I'd also like to mention that we held a World Cafe community meeting last month in December, where we brought together several community stakeholders to provide input on possible strategies for each of these high-level objectives. Our participants included representatives from managed care plans.

66
00:08:58.340 --> 00:09:03.720
Board Room: Our county partners, Community-based organizations, city staff.

67
00:09:04.010 --> 00:09:08.549
Board Room: As well as both steering committee and community members.

68
00:09:13.370 --> 00:09:19.219
Board Room: We've put together a video that hopefully conveys the depth and significance of this work.

69
00:09:19.350 --> 00:09:25.739
Board Room: As well as highlighting some of the experiences of our community steering committee members. I'll play it for you now.

70
00:09:37.020 --> 00:09:50.089
Board Room: So you're at 104 over 65, and your pulse is 87. So you want this one ideally, to be under 120. So you're in the normal range, which is great. Okay, he's just gonna register you, and then we're gonna have to have a seat on that side.

71
00:09:50.850 --> 00:09:53.579
Board Room: I was fast, I didn't even feel it.

72

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00:09:53.790 --> 00:10:06.790
Board Room: I think we all want the same thing. We all want our communities to be healthy. We want to see our children grow. We want to see our families happy and thriving.

73
00:10:07.860 --> 00:10:31.880
Board Room: To have a community health improvement plan that's based off of a community health assessment really allows us to be grounded in what is most important, not just to the city, but also to the community. We all took that, honestly. And define the intentions of where we want to go together in the next 3 to 5 years. If we want some of these objectives to be met.

74
00:10:31.880 --> 00:10:52.080
Board Room: kind of hinges on the strength of the partnerships. JSI was brought in basically to be facilitators in this space. I think a lot of what we do is hear from all these different perspectives, whether it's a community survey, or a focus group, or a community steering committee meeting, talking with the staff at City of Berkeley, and then synthesizing it and thinking about, okay, how do we talk about what happens next?

75
00:10:52.080 --> 00:11:02.239
Board Room: It was a learning process for the steering committee, understanding the data that was available, what resources and programs were already in place. What objective

76
00:11:02.240 --> 00:11:19.939
Board Room: do we have within the workforce domain? It's been amazing to see the really detailed underpinnings of what it is to build and to, you know, develop a city, but also, how do you make change in a city? How do you uproot the things that no longer work, and how do you plant the things that you hope will? And that's a really, really,

77
00:11:20.050 --> 00:11:31.080
Board Room: meaningful process, and not one that I had been able to kind of participate in before.

78
00:11:31.190 --> 00:11:45.620
Board Room: One of our core values for the city, this department, and for the public health division is that community voice, is that community drives the things that we develop, the programs we develop to serve them.

79
00:11:45.860 --> 00:12:07.469
Board Room: We talk in public health about how health is connected to so many different things, but being a part of this process, we really saw

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what that connection looked like, where all of the health priorities that rose to the top were behavioral health, community safety, environmental health, and housing, and none of those are, like, directly physically health-related, but are all interconnected to how people… what people feel like they need to be able to thrive here.

80
00:12:09.640 --> 00:12:26.020
Board Room: The focus areas this process identified isn't something that just the health department can work on in isolation. It offers an opportunity to invite other departments and other community-based organizations to join us in this journey.

81
00:12:26.370 --> 00:12:37.709
Board Room: these relationships and these collaborations are going to be even more important moving forward. We are definitely going to need the support of council and city leadership.

82
00:12:38.110 --> 00:12:42.639
Board Room: And we're definitely gonna need the support of our community members.

83
00:12:43.780 --> 00:12:57.079
Board Room: In working side-by-side with some of the folks across departments in the City of Berkeley, there is an extra level of passion and dedication that I honestly will say, to be totally open, wasn't really expecting.

84
00:12:57.540 --> 00:13:09.789
Board Room: What I saw was people continually inspired by… by their work. And how it kind of took a life of its own. It's energizing to know that they're really receptive to the ideas that we were bringing forward.

85
00:13:10.170 --> 00:13:29.140
Board Room: I just feel like my voice is heard, as we've been discussing, like, you know, how are we going to integrate this? What do we have to consider? Is this going to affect, a different demographic that we didn't think about? I love that I can see the tangible results coming through. I think, especially the way

86
00:13:29.260 --> 00:13:34.129
Board Room: the world is right now. You can feel really powerless.

87
00:13:35.630 --> 00:13:40.070
Board Room: And I would say yes and get involved locally.

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88
00:13:40.220 --> 00:13:45.789
Board Room: These kinds of projects can help you build community and have impact.

89
00:13:46.560 --> 00:14:05.839
Board Room: We need to center community members in order to understand the story. And then the second part is actually to understand what the right solutions are. And that was really important for us in the centering, was that community members be at the table to say, okay, so if we think the problem is displacement, for instance, in Berkeley, what are potential solutions?

90
00:14:05.870 --> 00:14:24.870
Board Room: We want to come up with ideas that are actually going to make a difference and move the needle on some of these issues, but we want to do it in the realm of what's possible, and not just say, this is what should happen, and demand that the city do something, but rather have this dialogue with people who are really embedded in these systems so that they're more feasible.

91
00:14:24.870 --> 00:14:38.740
Board Room: We really need to remain attuned to what the community's needs are, because they're going to be evolving, they're going to be dynamic, and we need to be able to be dynamic with that. The only way to do that

92
00:14:38.740 --> 00:14:40.660
Board Room: Is to listen to them.

93
00:14:41.320 --> 00:14:55.269
Board Room: As health assessments and health improvement plans become more of a standard part of the planning cycle in California communities, creating that space for trusting problem solving together is sort of the essential thing.

94
00:14:55.370 --> 00:14:59.850
Board Room: I just think that it's really important that if you want to make that change, you gotta be a part of it.

95
00:15:00.240 --> 00:15:09.539
Board Room: And that's why I love Berkeley so much, where there is such huge sense of community and commitment to one another. When a group of people fall down, there's another group who's trying to lift them up.

96

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00:15:37.630 --> 00:15:40.189
Board Room: Thank you, thank you.

97
00:15:41.130 --> 00:15:42.930
Board Room: We'll go on to our next slide, please.

98
00:15:59.010 --> 00:15:59.839
Board Room: There we go.

99
00:16:01.000 --> 00:16:08.299
Board Room: If you've been in Berkeley for a while, you know we used to produce a health status report every few years. The last one was back in 2018.

100
00:16:08.490 --> 00:16:15.519
Board Room: I'm happy to report that we'll have a new health status report coming in early 2027, and we'll be reporting back at that time.

101
00:16:15.930 --> 00:16:26.279
Board Room: I mean, while we will have a written report, we're moving away from the static reports. Our goal is no more documents sitting on a shelf. The data changes, and our community wants updates.

102
00:16:26.550 --> 00:16:42.049
Board Room: And they also want accountability through measurable outcomes. That means real-time access to data on health, health equity, other related data that anyone, including policy makers, nonprofits that are writing grants, or residents, can click on and access data at any time.

103
00:16:42.300 --> 00:16:48.049
Board Room: At this point, we want to give you a sneak peek demo of a public-facing dashboard that will go live this fall.

104
00:16:48.300 --> 00:16:56.610
Board Room: The reason that we're demoing this as part of this demonstration is this will be where we'll be reporting out progress on the CHIP activities as well.

105
00:16:56.790 --> 00:17:09.690
Board Room: I'm gonna show you some data that's, again, show you data that will be included in the dashboard, just to give you an idea of what will be available. I'll talk a little bit about what we're presenting, but

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106
00:17:09.920 --> 00:17:18.589
Board Room: The purpose of tonight isn't really to go into depth and have a detailed analysis for you on what this shows. It's really to give you an idea of what's coming.

107
00:17:18.920 --> 00:17:23.149
Board Room: And so with that, we're going to, Moved to…

108
00:17:24.839 --> 00:17:29.859
Board Room: The community… oh, we're logging in right now to the dashboard demo.

109
00:17:42.570 --> 00:17:43.559
Board Room: Here we go.

110
00:17:52.390 --> 00:17:53.070
Board Room: I'm sharing.

111
00:18:07.260 --> 00:18:08.090
Board Room: There we go.

112
00:18:08.240 --> 00:18:09.090
Board Room: So…

113
00:18:09.350 --> 00:18:25.790
Board Room: Once live, this is the first page that residents will see when they log into the dashboard. You'll see there's a table of contents, an introduction, and then the most significant part will be the key findings. And again, this data is taken directly out of our community health assessment.

114
00:18:26.330 --> 00:18:29.290
Board Room: For example, if we take a look at key finding number one…

115
00:18:29.950 --> 00:18:32.490
Board Room: We'll scroll down to the actual chart.

116
00:18:32.570 --> 00:18:50.399
Board Room: You'll see that Berkeley has a lot to be proud of. Our physical environment, our diversity, and our local partners are world-class. Generally, our health data, is impressive, but as you can see,

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Berkeley consistently outpaces both Alameda County and the rest of the state in life expectancy.

117
00:18:51.610 --> 00:19:00.120
Board Room: I want to just take a second and just show you, if you hover over, any part of this, it'll drill down a little bit, it'll tell you exactly

118
00:19:00.330 --> 00:19:11.149
Board Room: Where Berkeley is compared to, other, other communities, or you'll see in future charts here other information. Just makes it easier, because some of the charts get kind of confusing.

119
00:19:11.910 --> 00:19:14.049
Board Room: If we can go on to finding number 2.

120
00:19:18.780 --> 00:19:24.020
Board Room: So we'll scroll down here. This one is about racial disparities, and as you can see.

121
00:19:24.250 --> 00:19:37.629
Board Room: While our general life expectancy is good in Berkeley, actually extremely good, the data makes it abundantly clear that there are disparities that we have to talk about as we design interventions.

122
00:19:37.770 --> 00:19:47.789
Board Room: It's true, we are a vanguard of social movements, yet our health data shows that historical and structural identity issues are still deeply baked into our outcomes.

123
00:19:48.000 --> 00:19:52.699
Board Room: So you can see here we have, life expectancy by, by race.

124
00:19:54.220 --> 00:19:59.610
Board Room: If you live in Berkeley… I'm sorry, we'll go on to the, let me just stop there for a second.

125
00:20:01.150 --> 00:20:04.340
Board Room: This particular one shows life expectancy by race.

126
00:20:04.490 --> 00:20:10.750
Board Room: If we go to the next one, we'll go on to finding number 3, we'll also show life expectancy

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127
00:20:11.000 --> 00:20:12.930
Board Room: By geographic location.

128
00:20:17.800 --> 00:20:24.689
Board Room: And so, if you live in Berkeley Hills, for example, you'll see that your life expectancy is 91 years.

129
00:20:26.000 --> 00:20:32.129
Board Room: If you also live in certain census tracts in South or West Berkeley, that number drops to, 78.

130
00:20:32.510 --> 00:20:33.880
Board Room: 78 years.

131
00:20:34.160 --> 00:20:39.939
Board Room: A 13-year-old difference based on zip code is not just a statistic, it's a call to action.

132
00:20:40.350 --> 00:20:48.249
Board Room: If we can't measure it, we can't track it, we can't change it. Our biggest opportunity for progress lies exactly where the outcomes are the worst.

133
00:20:48.970 --> 00:20:58.139
Board Room: And we'll exit our demo. So again, just wanted to give you a quick snapshot that that will be available, and we'll send you information as soon as it's live.

134
00:20:58.450 --> 00:21:06.900
Board Room: We can go on to our next slide. I have another slide that will be built into the dashboard, and this particular one is slide 9.

135
00:21:10.130 --> 00:21:19.569
Board Room: Slide 9 is another example of data that will be front and center on our new dashboard, because it highlights the growing fact… a growing fact that we cannot ignore.

136
00:21:19.710 --> 00:21:35.560
Board Room: between 20 and 22 and 2024, drug overdoses and self-harm suicide rose to, into the top 10 leading causes of death, for our neighbors. And you'll see here, it is number 1, 2, 3, 4… number…

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137
00:21:35.850 --> 00:21:45.159
Board Room: number 6. That's going up from previous years, where it had, been in the bottom of the top 10, rose to, number 8, and now it's up to number 6.

138
00:21:45.570 --> 00:21:59.179
Board Room: We know why this is happening. Along with the deep emotional trauma caused by the pandemic in our society, social isolation, economic stress, and everything going on in the world today have created the perfect storm for many of our residents.

139
00:21:59.310 --> 00:22:04.290
Board Room: That's exactly why the community identified behavioral health as a priority in the CHIP.

140
00:22:04.420 --> 00:22:12.659
Board Room: It's also why we must continue our efforts to integrate behavioral health and physical health at all levels. We need a multi-pronged approach.

141
00:22:13.170 --> 00:22:18.839
Board Room: Including wellness programs, easier access to care, and tighter bonds with our community partners.

142
00:22:18.970 --> 00:22:28.930
Board Room: In a time of declining revenue, it's more important than ever to be good stewards of public resources. Partnering with and empowering our community is more important than ever.

143
00:22:29.790 --> 00:22:30.609
Board Room: Next slide.

144
00:22:32.010 --> 00:22:40.869
Board Room: So, implementation is really where the rubber meets the road. You're forming advisory groups for the four pillars, including housing.

145
00:22:41.040 --> 00:22:44.810
Board Room: Environmental health, behavioral health, community safety.

146
00:22:45.000 --> 00:22:55.530
Board Room: They'll help us shape the actions that we're going to take. You'll see right now, and this is outlined in the CHIP document, we're in the preparation stage, and as I said, that's where we've been selecting

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147
00:22:55.630 --> 00:23:11.150
Board Room: members of the community, stakeholders, as you see, some of those objective areas involve entirely different departments than the city, but we're all connected. Environmental health, for example, having green spaces, having safe

148
00:23:11.360 --> 00:23:15.279
Board Room: Areas for the community, all part of the public health strategy.

149
00:23:16.960 --> 00:23:22.710
Board Room: And finally, I want to make one important observation here.

150
00:23:22.970 --> 00:23:33.670
Board Room: I've worked in a variety of communities over the last 35 years, and there's really something that's unique to Berkeley, and you all might know this, but being here for about a year and a half now.

151
00:23:33.790 --> 00:23:46.620
Board Room: It really, it really jumps off the page at me. I'm hearing in multiple venues, our residents want to be involved, they want to help, they even want to roll up their sleeves, and they demand to be part of the process. They demand that.

152
00:23:47.090 --> 00:23:55.430
Board Room: And while it would be easy for us to take this… very easy for us to take this roadmap that the committee put together for us, lock ourself in a room for a nice…

153
00:23:55.550 --> 00:24:02.460
Board Room: launch and strategic planning process and come back and deliver you a strategic plan. That's not what the community demands of us.

154
00:24:02.660 --> 00:24:21.419
Board Room: What we're gonna do at this point is we will be working with the focus groups during implementation, and we'll develop very specific, actionable, measurable outcomes that we can implement in our community with their help, that have value to the community members, and that they,

155
00:24:21.480 --> 00:24:24.080
Board Room: That they also perceive as having value.

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156
00:24:25.140 --> 00:24:44.560
Board Room: So from, actually from December until July, we're in that preparation stage. The reason that we put action in July is, we are going to be working on this before July, believe me, but we also want to make sure that we get through the budget process before we really, really, formalize structures and bring groups together, because we are expecting

157
00:24:44.670 --> 00:24:49.279
Board Room: You know, there'd be difficult decisions, and we want to make sure that, again, that we're good stewards of money.

158
00:24:49.510 --> 00:24:57.630
Board Room: We realize that we're… that some of this will, as I said, will involve community, rolling up their sleeves and making things happen.

159
00:24:59.490 --> 00:25:00.360
Board Room: Alright.

160
00:25:00.720 --> 00:25:02.780
Board Room: And so, with that,

161
00:25:06.270 --> 00:25:11.780
Board Room: My last page… So the journey towards a healthy.

162
00:25:12.100 --> 00:25:24.560
Board Room: Equitable, safer Berkeley requires more than just observation. As we said, it requires partnerships. We invite the Council to walk this path with us, and we'll definitely keep you informed and invite you to participate every step of the way.

163
00:25:24.880 --> 00:25:35.319
Board Room: We will be fueled by the courage to change the status quo, and to quote James Baldwin, not everything that is faced can be changed, and nothing can be changed until it's faced.

164
00:25:35.980 --> 00:25:37.849
Board Room: So we'll go to slide 11, please.

165
00:25:40.880 --> 00:25:52.980
Board Room: So at this time, we really have two items left. We'll take any questions, comments that you have at this point, or discussion, and

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then we'd like to, conclude by recognizing members of the steering committee.

166
00:25:53.610 --> 00:25:54.630
Board Room: Thank you, Mayor.

167
00:25:55.090 --> 00:26:01.660
Board Room: Thank you so much, Director. And so I want to ask if my colleagues have any questions.

168
00:26:05.340 --> 00:26:14.390
Board Room: I'm not sure if our parliamentarian is working, so I see that Councilmember Draco is trying to select this, so give me a second, let me reset it, and you can start while I…

169
00:26:15.020 --> 00:26:16.179
Board Room: Reset it.

170
00:26:16.420 --> 00:26:17.940
Board Room: Thank you so much.

171
00:26:19.010 --> 00:26:23.749
Board Room: Thank you, Scott and Tanya, for your presentation.

172
00:26:24.180 --> 00:26:32.290
Board Room: I have a few questions. I will, to save time, ask them in rapid fire. I think they go from…

173
00:26:32.830 --> 00:26:43.470
Board Room: more specific to kind of more meta. My first question is on page 6, under objectives, I see that you called out

174
00:26:43.980 --> 00:26:47.219
Board Room: I, I assume… These are just…

175
00:26:48.030 --> 00:26:52.310
Board Room: Examples, and there are other,

176
00:26:52.480 --> 00:26:57.680
Board Room: sub-objectives, say, under community safety, I imagine.

177

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00:26:58.420 --> 00:27:12.640
Board Room: there would be more than just improving walkability and walking safety that could be placed in that bucket. Just not sure exactly what page you're referring to. Sorry, page 6 of the presentation. Sure.

178
00:27:13.890 --> 00:27:23.559
Board Room: On the life expectancy slide, which… I think is not in…

179
00:27:23.720 --> 00:27:27.410
Board Room: The slide deck, but it was in the report.

180
00:27:28.270 --> 00:27:41.159
Board Room: I wanted to see… you may or may not be able to talk about this, it may be too granular, but I also noticed an area in, up or downtown,

181
00:27:41.990 --> 00:27:47.089
Board Room: below South Side of campus. That was…

182
00:27:47.860 --> 00:27:55.390
Board Room: comparatively low to life ex… in life expectancy as well. I was wondering if you could talk about… Why?

183
00:27:55.580 --> 00:28:02.580
Board Room: I was curious how the focus group was selected.

184
00:28:02.840 --> 00:28:09.810
Board Room: And what equity screens, Were incorporated in the selection process.

185
00:28:10.320 --> 00:28:18.750
Board Room: As well as, well, you talked about how you would continue to engage with the focus group, but would it be the same focus group

186
00:28:18.960 --> 00:28:21.230
Board Room: Or… a different one.

187
00:28:22.220 --> 00:28:27.109
Board Room: On page 19 of the report,

188
00:28:27.450 --> 00:28:33.089

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Board Room: I was wondering how CHIP can target interventions to communities that are most affected.

189
00:28:33.670 --> 00:28:38.969
Board Room: And how will the root causes of these health disparities be addressed?

190
00:28:39.200 --> 00:28:50.940
Board Room: And… Lastly, I wanted to see if, I mean, you… You spoke to… how,

191
00:28:52.010 --> 00:29:00.549
Board Room: Many of these, the challenge, or the… The interventions,

192
00:29:00.810 --> 00:29:14.570
Board Room: may not relate, may not be specific within the community health bucket. They may be informed by other disparities and barriers, whether it's within housing or other departments.

193
00:29:16.070 --> 00:29:20.779
Board Room: What is your vision and plan for…

194
00:29:21.170 --> 00:29:26.100
Board Room: A cross-cutting process, an interdepartmental process.

195
00:29:26.330 --> 00:29:27.540
Board Room: around this.

196
00:29:27.740 --> 00:29:35.000
Board Room: And how will you be keeping the Council apprised at various inflection points?

197
00:29:39.500 --> 00:29:52.500
Board Room: Thank you, Councilmember Jacob. Those are a lot of questions, so hopefully we can address all of them. So, first I'd like to address the question that you had around the community safety objective.

198
00:29:52.550 --> 00:30:04.939
Board Room: So this is a high-level objective that our community steering committee identified, that really rose to the surface from the data that was gathered. And, the idea is to…

199

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00:30:05.320 --> 00:30:13.090
Board Room: identify sub-objectives and strategies that will help address this. And also, since we are going to

200
00:30:13.090 --> 00:30:36.710
Board Room: undergo a process of revisiting, our planning and priorities every 3 years. My assumption would be that the high-level objectives would, under each of these areas, would evolve over time. And, even the priority areas themselves may evolve over time, depending on how the communities

201
00:30:36.740 --> 00:30:48.589
Board Room: needs and priorities change. So, at this point, improving walkability and walking safety was something that really rose to the level of importance for them.

202
00:30:48.680 --> 00:30:56.510
Board Room: But it could be… that could change, when we revisit, This area, the future.

203
00:30:58.590 --> 00:31:01.160
Board Room: Do you want to take the question about life expectancy?

204
00:31:01.460 --> 00:31:08.690
Board Room: Yeah, so… Drilling down into those specific things are what the next steps will be.

205
00:31:08.950 --> 00:31:17.910
Board Room: I don't have the answer for how the focus groups were selected. If you're referring to the… to the community cafe.

206
00:31:18.040 --> 00:31:20.970
Board Room: That was open to the… to everybody.

207
00:31:21.110 --> 00:31:22.140
Board Room: For that one.

208
00:31:24.840 --> 00:31:41.610
Board Room: I will add that, when the consulting firm conducted the focus groups that were part of the health assessment process, they did identify priority populations to conduct those focus groups. And again, I think they conducted a total of four.

209

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00:31:41.610 --> 00:31:50.570
Board Room: I think one of them was with transitional age youth. I think one of them might have been, with African American and Latino.

210
00:31:50.590 --> 00:31:54.220
Board Room: And I can't recall the other two.

211
00:31:56.260 --> 00:31:57.619
Board Room: But we can get back to you on that.

212
00:31:59.880 --> 00:32:07.170
Board Room: Yeah, and, to your question about how we will ensure, other departments, other stakeholders.

213
00:32:07.300 --> 00:32:13.189
Board Room: That's the process that we'll be going through from now until July, is looking at each of these areas.

214
00:32:13.270 --> 00:32:32.530
Board Room: And I want to also mention that, the steering committee gave us a bunch of ideas, for specific solutions or interventions, but they didn't dictate it to us. So, our challenge now will be is pulling together the different stakeholders, going through the data.

215
00:32:32.870 --> 00:32:44.140
Board Room: And then figuring out actionable, actionable objectives, solutions, as they referred to in the video, that we can implement within the financial restraints that we have.

216
00:32:45.190 --> 00:32:56.139
Board Room: because this process is going to be so transparent, we're planning at a minimum, of annual updates to Council, the first one being, January, February-ish of 2027.

217
00:32:56.380 --> 00:33:02.429
Board Room: The selfish me wants to make sure that whatever we commit to doing, we're gonna get done and we're gonna deliver, for you.

218
00:33:02.670 --> 00:33:08.789
Board Room: Again, this is different, for, most, health departments around the state.

219
00:33:08.910 --> 00:33:21.289

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Board Room: Where they do plop down a plan on the table, and that's the end of it. So because we are going to be developing this community, we're going to be very transparent, we're going to be very measurable, so that we can demonstrate our value.

220
00:33:21.640 --> 00:33:26.849
Board Room: We absolutely have to be very, very focused on what we do, make sure we can accomplish that.

221
00:33:29.710 --> 00:33:33.279
Board Room: I think that was the last question, but make sure… let us know if there's anything.

222
00:33:33.700 --> 00:33:41.249
Board Room: Yeah, just wanted to see, if you can… drilling down, I…

223
00:33:41.930 --> 00:33:52.880
Board Room: Certainly South and West Berkeley were called out as, communities of concern, but I did notice a, also a…

224
00:33:53.260 --> 00:34:01.599
Board Room: reduced lifespan in up or downtown, and I wanted to see if, at least within other parts of Berkeley.

225
00:34:01.720 --> 00:34:07.149
Board Room: I wanted to see if you can speak to why that may be.

226
00:34:09.739 --> 00:34:10.489
Board Room: Yep.

227
00:34:10.929 --> 00:34:11.919
Board Room: I don't think…

228
00:34:12.159 --> 00:34:18.639
Board Room: I don't think we're to the Y yet. We're to the… we're to the, this is… this is the data and what it is.

229
00:34:18.810 --> 00:34:21.640
Board Room: I, I think the… there's a…

230
00:34:22.090 --> 00:34:27.329

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Board Room: There's an infinite number of Y's, you know, some of them, For example, we have…

231
00:34:27.639 --> 00:34:36.080
Board Room: data on, different, health conditions. Asthma was a big one, that, that we've looked at that, that comes out.

232
00:34:36.270 --> 00:34:38.369
Board Room: In the community health assessment.

233
00:34:38.620 --> 00:34:47.989
Board Room: And so as we look at asthma, we can also look at asthma rates in different areas, and then we can also start to match that up, with the outcomes that we want.

234
00:34:48.170 --> 00:35:03.000
Board Room: We're fortunate that we have epidemiologists on our staff, you know, that will work with us on that, but but that sort of deep analysis and what does it really mean to the community, we need to talk with the community about that, you know, and involve them in that process.

235
00:35:05.800 --> 00:35:09.029
Board Room: Thank you. Moving on to Councilmember Bartlett.

236
00:35:10.600 --> 00:35:25.489
Board Room: Well, thank you, and thank you for your work here. I… I recall, this is… I recall, the CHIP was, conceived a few years ago, when we funded the Health Equity Innovation Zone. I think the city manager was here then before he went to Emeryville.

237
00:35:25.740 --> 00:35:26.829
Board Room: I think I remember that.

238
00:35:27.100 --> 00:35:28.880
Board Room: So it's wonderful to see in action.

239
00:35:29.030 --> 00:35:35.390
Board Room: Wonderful. Amazing. Question, looking at the leading cause of death,

240
00:35:36.620 --> 00:35:42.429

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Board Room: So I guess diabetes is not on here, right? But is that not a cause of death, or is it just a cause of… cost?

241
00:35:46.810 --> 00:35:52.739
Board Room: Diabetes number 3? Is that what you're asking about? I said, I said heart disease. Oh, I'm sorry.

242
00:35:53.120 --> 00:35:54.310
Board Room: Am I missing it?

243
00:35:55.870 --> 00:35:59.739
Board Room: Not in the top 10. It's not there. Wow, that's amazing. That is amazing.

244
00:36:00.400 --> 00:36:04.330
Board Room: Well… Very healthy city. You know, the, the,

245
00:36:05.060 --> 00:36:14.109
Board Room: You know, the work is incredible that we do here in Berkeley. We are the healthiest city, and of course, as you mentioned, the disparities are super entrenched, and we've…

246
00:36:14.260 --> 00:36:24.250
Board Room: Taking many steps to address them, and this is part of a long-running effort by, you know, your team in my office to get at some new ways of approaching

247
00:36:24.420 --> 00:36:33.949
Board Room: the, the equity elements here. And so I was curious, a couple things I wanted to ask you about, more just, just pegs for your memory, I guess.

248
00:36:34.160 --> 00:36:48.539
Board Room: Will there be… is it possible to have a dashboard assigned to this stuff, you know, related to the budget? I know Councilmember Blackaby has been working on something like this, some sort of dashboard that's readily available for people to see, or progress.

249
00:36:48.920 --> 00:36:52.420
Board Room: And, does, another element…

250
00:36:52.550 --> 00:36:57.260
Board Room: was curious about was, alignment with Medi-Cal, Medicaid.

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251
00:36:57.790 --> 00:37:02.519
Board Room: Right? Despite the cuts of those programs recently, right? However that works.

252
00:37:02.640 --> 00:37:08.710
Board Room: And then, I guess with the… it seems that we put a lot of,

253
00:37:08.900 --> 00:37:15.750
Board Room: And rightfully so, we leverage our local talent, which is immense here in Berkeley, for their ideas, in…

254
00:37:16.040 --> 00:37:20.940
Board Room: So, the… I guess the… Is there a strong decision-making structure?

255
00:37:21.200 --> 00:37:26.670
Board Room: So, they would make recommendations to your office, or, you know, do you have something in place to…

256
00:37:28.930 --> 00:37:46.289
Board Room: Yeah, in terms of the decision-making structure for how the specific, solutions will be identified, that will be through the community. There'll be an oversight, and we've approached the community health commission, to be part of that process with us.

257
00:37:46.470 --> 00:37:48.249
Board Room: And,

258
00:37:49.880 --> 00:37:57.359
Board Room: At this point, there's also calling for another steering committee to be established as we get further down the road.

259
00:37:57.890 --> 00:38:01.700
Board Room: In terms of the Medi-Cal,

260
00:38:01.830 --> 00:38:04.550
Board Room: the Medi-Cal implications, or how this fits.

261
00:38:04.670 --> 00:38:22.329
Board Room: It absolutely fits because, when I said integration with primary care and behavioral health, that's really the huge change that

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we're seeing right now in the Medi-Cal world, where they're actually requiring that. So this is the first year that there's been a requirement on both sides, in both silos.

262
00:38:22.440 --> 00:38:31.470
Board Room: to work together with the… you're probably familiar with the 3-year action plan that we have to submit and get approved from the Mental Health Commission.

263
00:38:31.630 --> 00:38:35.129
Board Room: That has a ton of data, a ton of information.

264
00:38:35.530 --> 00:38:42.770
Board Room: before this cycle, that process never touched this process. They were done in complete isolation.

265
00:38:42.910 --> 00:38:53.010
Board Room: The state's now requiring, as Tanya mentioned, they're requiring us to work together, and that's all around, our ability to bill and be reimbursed for Medi-Cal, and do that.

266
00:38:53.010 --> 00:39:04.739
Board Room: So… so yeah, we absolutely see this, and as we look at whole-person care, you know, working together, around Medi-Cal. Also, the health plans are very much involved in this process, and they're required to be.

267
00:39:04.760 --> 00:39:13.879
Board Room: And which is great, because they're responsible for paying for, you know, for a lot of this. And so the state, I think, was very,

268
00:39:13.910 --> 00:39:20.049
Board Room: Forward-thinking when they put the requirements together for this, requiring the health plans to be at the table with us.

269
00:39:20.260 --> 00:39:24.769
Board Room: Requiring behavioral health to be part of the process, and then actually

270
00:39:24.980 --> 00:39:37.830
Board Room: a small pot of money out of the behavioral health fund will come to Public Health to help convene and coordinate all of this work. That's wonderful. That's so great. And, let's just leave you with a couple points just to keep in mind.

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271
00:39:37.950 --> 00:39:43.710
Board Room: since Alzheimer's is… dwarfs so many of the other causes of death here.

272
00:39:44.040 --> 00:39:50.260
Board Room: The… it'd be great to align with the senior centers, and… because there have been some ideas about ramping up

273
00:39:50.550 --> 00:39:59.910
Board Room: the senior centers into more than just meeting places for Zumba, but for, you know, real healthcare delivery, community health delivery vehicles to the community.

274
00:39:59.910 --> 00:40:11.440
Board Room: And then the heart disease and hypertension, which factors really greatly in my district, as well as, two other persons in the district in the body here. You know, this…

275
00:40:11.460 --> 00:40:20.590
Board Room: Someone on your body should be, I guess, aware of the city council's measures that exist, like healthy checkout.

276
00:40:20.780 --> 00:40:25.159
Board Room: Right, and so… that, that, that, that program is subbed from lack of funds.

277
00:40:25.580 --> 00:40:35.769
Board Room: But we know that, the stores in the communities where the heart disease is the highest has the highest concentration of, of heart disease making food.

278
00:40:35.960 --> 00:40:44.800
Board Room: Right, all over the place, right? Not to mention vapes and etc. So it'd be great to align some of these resources, to help implement

279
00:40:45.140 --> 00:40:54.110
Board Room: those health-making items I already have that are just suffering from lack of funding, right? And then…

280
00:40:54.930 --> 00:40:56.820
Board Room: That's all I can think of. Thanks very much.

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281
00:40:57.210 --> 00:40:58.029
Board Room: Thank you.

282
00:40:58.950 --> 00:41:01.789
Board Room: Thank you. Moving on to Councilmember Blackby.

283
00:41:01.900 --> 00:41:09.869
Board Room: Thanks, Madam Mayor, and thank you, Director Gilman, and Deputy Director Bustamonte, for the great report, and thanks to all the team for putting this together.

284
00:41:10.050 --> 00:41:26.379
Board Room: Councilmember Lunapard and I were commenting about how great the video was, but everything is just really… I mean, we really appreciate the effort and the care that went into this. I just had a couple of questions, or comments and then questions. One is, and Councilmember Bartlettman mentioned, I really love the focus here on measurable

285
00:41:26.380 --> 00:41:40.710
Board Room: outcomes in your comment. Again, if you can't measure it, you can't manage it, you can't track it, and so that's so important. And so, having that on the dashboard, and having that basis of information that we can then say, are we improving or not, is really important, I appreciate that.

286
00:41:40.710 --> 00:41:48.980
Board Room: Also, your, focus on… this is sort of the first step, this is the… the plan, but so much of this is about the implementation.

287
00:41:49.250 --> 00:42:04.779
Board Room: And what are we gonna actually do in each of these buckets to sort of move the needle? I'm really excited to see what comes back as you move into that phase, so that it really becomes an action plan, and there's really things and initiatives that you're tracking back and seeing what happens.

288
00:42:04.990 --> 00:42:20.059
Board Room: And then last, as other people have commented how sobering some of those deltas are on the neighborhood outcomes, especially life expectancy. I think we all kind of knew that, but to see that so clearly and understand how big the deltas are, that's obviously something we have to really focus on and improve.

289

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00:42:20.400 --> 00:42:25.719
Board Room: So, first question is, you know, community health, big topic.

290
00:42:25.840 --> 00:42:39.829
Board Room: very broad, some of which we can have an impact on here locally, some of which are factors that are far beyond our control. I'm wondering, as you put together, sort of, these goals and the objectives, how did you think about that? Were there, sort of.

291
00:42:39.970 --> 00:42:45.030
Board Room: Did you intentionally look at some things that were like, oh, that would really be great, but we actually just don't think we can…

292
00:42:45.070 --> 00:43:01.280
Board Room: do it? Are there, you know, categories of disease, or you'd mentioned drug addiction things before. So, when you came up with those goals, I'm wondering if you… if you also looked at other things and sort of said, well, you know, we're not going to put this in the plan, because this isn't something we can do at the community level. Just curious about that process.

293
00:43:01.660 --> 00:43:22.509
Board Room: Yeah, I mean, as I mentioned before, I think the four priority areas, really rose to the surface, and they were the areas that the steering committee, identified as, ones that we want to focus on, at least for the next three years. And, you know, there were a myriad of other findings and,

294
00:43:23.230 --> 00:43:31.960
Board Room: And data that was identified in the health assessment, but, you know, we wanted to keep things, again, reasonable and feasible.

295
00:43:32.020 --> 00:43:47.959
Board Room: And so the steering committee, along with some guidance from city staff, identified those areas as ones to tackle first. And, I think as we move forward, in the action phase, that will be…

296
00:43:48.500 --> 00:43:49.729
Board Room: That will be…

297
00:43:50.020 --> 00:44:06.920
Board Room: it will be critical to really kind of hone down, actions that are feasible and realistic, and things that are within the span of control, and be able to relay that to the community. Like, this is what, the city has

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298
00:44:06.920 --> 00:44:18.509
Board Room: control over, and this is something that we can move forward on in collaboration with the county, or with the state, or with other community-based organizations, and also be transparent about what's outside of our control.

299
00:44:19.520 --> 00:44:22.320
Board Room: Just to follow up on that, you're right, there's…

300
00:44:22.460 --> 00:44:32.000
Board Room: pretty much anything anymore is called public health. You know, some communities are declaring gun violence a public health crisis, so it's huge, it's huge, and what I love about…

301
00:44:32.050 --> 00:44:46.889
Board Room: what this process, accomplished, is that the community told us what the top four are, you know, and, they, they had quite a detailed process, and they said, this is, this is where we want you to focus, you know, to get started.

302
00:44:47.190 --> 00:45:04.980
Board Room: And so, the ball's in our court. Great. You referenced, kind of the sneak peek, I think, page 18 in the actual blueprint, where it's sort of the first look at, you know, one strategy in each category to advance, and then, you know, so…

303
00:45:05.300 --> 00:45:12.239
Board Room: Resources, workforce belonging across all four of those different subjects, and it, again, it's kind of exciting to see some of those…

304
00:45:12.410 --> 00:45:21.599
Board Room: first, you know, first looks at maybe what some of the actions might be. When do you think, you know, in the timeline might be the point where we might see what the first

305
00:45:21.600 --> 00:45:33.090
Board Room: kind of next version of that implementation plan is. You know, the next… because again, to me, that's where the rubber hits the road. It's the part that's really exciting. It's like, what are we actually doing? When do you think that we might…

306
00:45:33.180 --> 00:45:34.630

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Board Room: Just… see that.

307
00:45:35.740 --> 00:45:46.059
Board Room: You know, our goal was to start much sooner on that part than July, but because of budget discussions, that's why we're pausing just a little bit.

308
00:45:46.080 --> 00:45:56.239
Board Room: That said, we're still gonna keep working on this. So I would hope that by the time we come back in January of 2027, that we'll have some initial

309
00:45:56.330 --> 00:46:04.850
Board Room: initial, I want to keep calling them solutions instead of specific measurable objectives, but solutions that we can share with you. Okay.

310
00:46:05.170 --> 00:46:06.949
Board Room: And last question is,

311
00:46:07.060 --> 00:46:18.080
Board Room: given all that is, again, these are big, bold, some of these are big, bold goals. You know, how confident are you that, you know, we're gonna be able to move the needle on at least some of these, if not all these?

312
00:46:18.680 --> 00:46:20.299
Board Room: This is Berkeley.

313
00:46:20.420 --> 00:46:22.940
Board Room: Seriously.

314
00:46:23.100 --> 00:46:32.330
Board Room: I've never been in a community, and I've said this when the federal budget cuts were raining down upon us, and I'm like, oh my gosh, how am I even gonna run a public health department? I said.

315
00:46:32.450 --> 00:46:44.970
Board Room: I know if public health puts a call out to our community for help, that they will step up. And so, I'm confident if we do this right, and we let the community roll up their sleeves and get involved.

316
00:46:45.170 --> 00:46:48.879

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Board Room: And help figure out what it's gonna be that we will move the needle.

317
00:46:49.550 --> 00:46:52.049
Board Room: Well, thank you so much. It's really great work. Thank you.

318
00:46:52.440 --> 00:46:55.590
Board Room: Thank you. Other questions from my Council colleagues?

319
00:46:56.500 --> 00:47:20.779
Board Room: I have a few questions as well. So thank you all very much. I really appreciate just this report and everyone that did work. I know that these reports can take a very long time and involve a lot, a lot of work, so, thank you, and I especially appreciate the community engagement that you have intertwined throughout this process. I think that's really essential, and you're right, our community does demand that of us, so…

320
00:47:20.780 --> 00:47:28.179
Board Room: I… so I have some questions. I'm curious about what is the difference between the community health assessment in 2025 and the…

321
00:47:28.180 --> 00:47:34.930
Board Room: health status report, which I think you mentioned was the 2018, I just… there's a lot of different reports and plans and things, so…

322
00:47:36.210 --> 00:47:51.609
Board Room: So, the last health status report that our department put out in 2018, really focused on quantitative data and, data that's collected, both locally and from other,

323
00:47:51.690 --> 00:48:09.149
Board Room: state and regional reporting sources, and some of them also nationwide. And, the previous practice was that we put out a health status report, I think, about every 4 or 5 years, that, again, really focused on the quantitative, aspects of the community's health.

324
00:48:09.240 --> 00:48:16.799
Board Room: The health assessment is, more focused… well, it does take into account the quantitative data.

325
00:48:16.980 --> 00:48:34.539
Board Room: But it also, engages the qualitative aspect. So, the informant interviews, from community leaders, the focus groups, the

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community survey, all of that helped to really, build a more comprehensive story and picture of, of our health.

326
00:48:36.860 --> 00:48:44.330
Board Room: Thank you, yeah, that's helpful to understand. And then… I was also curious…

327
00:48:44.350 --> 00:49:03.809
Board Room: In the past, the city prioritized addressing chronic diseases like high blood pressure and heart disease, due to the serious racial and geographic, disparities between communities of color in South and West Berkeley versus the Hills. Is there a reason that this data and the strategies are not included in this plan?

328
00:49:07.550 --> 00:49:13.530
Board Room: Are you… when you say the… are you referring to the four priority areas? Or,

329
00:49:14.580 --> 00:49:28.049
Board Room: No, I think that in the past, they would show, like, disparities by these different diseases, by sort of the racial breakdown and also geographic breakdown. I think it's in here for cancer, maybe?

330
00:49:28.290 --> 00:49:32.909
Board Room: But… Yeah, I'd have to go back and look… actually…

331
00:49:33.070 --> 00:49:40.780
Board Room: Where that would be located is in the Community Health Assessment, and so that will be populating, we'll be bringing that forward.

332
00:49:41.370 --> 00:49:51.379
Board Room: But in terms of the priority areas, it doesn't mean we're not gonna work on that, but it just wasn't one of the main areas that they identified. I see.

333
00:49:52.830 --> 00:50:10.909
Board Room: But having that… if you're referring to having that data available, that data will be available. It will be available on the… on this… on this dashboard that you're showing us. Okay, great. Okay, I have some more comments, but I'll save them for later. Thank you. I… what I want to do now is open it up for, public comment.

334
00:50:11.590 --> 00:50:14.419

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Board Room: Is there any public comment on this item? Thank you.

335
00:50:19.930 --> 00:50:39.139
Board Room: Hi, good evening, Mayor Ishii and Council. I'm Andy Katz, I'm Chair of the Community Health Commission, and, speaking as an individual, because our Commission has, not had a chance to, meet since this, went live, but I would like to share that the, you know, we've received some interim presentations, and we've all seen that the

336
00:50:39.600 --> 00:50:44.610
Board Room: of housing, community safety, behavioral health, and,

337
00:50:44.750 --> 00:50:57.619
Board Room: environmental health, they bring new insights, and we should all take note and reimagine what kinds of solutions in the spirit of community prevention and empowerment we can have as a result of this work.

338
00:50:57.730 --> 00:51:17.159
Board Room: But let's think about what CHIP is and what it isn't in this format. Health in all policies is a really exciting approach to take, and I think that's what this is doing, is looking at really big issues that affect all of us, and highlighting how they affect our health.

339
00:51:17.430 --> 00:51:35.599
Board Room: There's a big value to aligning with the Future of Public Health program. We can access funding through the plans. There's a lot of benefits to the way the city's engaged with this. But the CHIP is so high level here. Moreover, it falls short of addressing

340
00:51:35.620 --> 00:51:44.279
Board Room: the strategic plan needs for the Public Health Division. So this leaves me with a deep concern that if CHIP would be intended to.

341
00:51:44.360 --> 00:52:00.299
Board Room: address prevention of chronic disease, prevention of infectious disease. It's clearly not doing that, as many of you have noted in your questions. It's critical for the City of Berkeley to improve and maintain programs that promote public health.

342
00:52:00.380 --> 00:52:02.070
Board Room: Through peer-to-peer education.

343
00:52:02.120 --> 00:52:11.119

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Board Room: Closing gaps in access to care, direct services like public health nursing, and an iterative assessment with a new kind of results-based accountability.

344
00:52:11.120 --> 00:52:30.360
Board Room: the city needs to move forward with that as well. So how will CHIP implementation be positioned to be successful across these pillars that are not traditionally implemented by HHES? I'll wrap up… Thank you. I'll wrap up with one more. You can, you can actually, if you have additional comments, you're welcome to send them to us, too. Thank you. Okay, thank you. Thanks.

345
00:52:34.220 --> 00:52:44.089
Board Room: Good afternoon, Council. I'm Tajitz-Baptiste, Community Health Commissioner. I'd like to firstly second, what my Commission colleague just, said.

346
00:52:44.320 --> 00:52:50.780
Board Room: And also to say that, the city's been given a mandate on this, this CHIP report. I think it, it fairly…

347
00:52:51.070 --> 00:52:56.610
Board Room: concisely spells out, what my fellow community members have

348
00:52:57.120 --> 00:52:59.640
Board Room: want around community health, I mean, that…

349
00:52:59.750 --> 00:53:03.400
Board Room: These four pillars that we've been given,

350
00:53:04.770 --> 00:53:09.040
Board Room: perfectly emblemize what our city is facing right now.

351
00:53:10.140 --> 00:53:24.310
Board Room: So also, the City of Berkeley now has both the data and the mandate to act. The Berkeley Wellness Blueprint makes clear that life expectancy in our city differs by as much as 13 years, depending on neighborhood. 91 years in the Berkeley Hills, and just 78 in South and West Berkeley.

352
00:53:24.720 --> 00:53:26.540
Board Room: That disparity is not accidental.

353

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00:53:26.840 --> 00:53:38.959
Board Room: It is a predictable result of housing instability, environmental justice, behavioral health gaps, and unequal neighborhood investment identified in the CHA, the Community Health Assessment, and elevated in this

354
00:53:39.130 --> 00:53:40.589
Board Room: Community Health Improvement Plan.

355
00:53:41.340 --> 00:53:56.970
Board Room: At the same time, California's future public health framework warns that fragmented, categorical, and historically underfunded systems cannot meet modern health challenges without stable investment in workforce, data infrastructure, emergency preparedness, communications, and community partnership.

356
00:53:58.000 --> 00:54:04.200
Board Room: Berkeley cannot adopt the language of equity without committing to the structural investments that make equity real.

357
00:54:04.480 --> 00:54:05.780
Board Room: This community health…

358
00:54:06.150 --> 00:54:13.739
Board Room: Improvement Plan explicitly states that words and reports alone do not create change without accountability or dedicated funding.

359
00:54:14.870 --> 00:54:27.350
Board Room: Therefore, I urge the City Council and City Manager to formally align budget, interdepartmental work plans, and performance metrics with the CHIP's two central goals. Closing the life expectancy gaps and increasing community power.

360
00:54:27.490 --> 00:54:33.989
Board Room: That means targeting resources to the neighborhoods with the worst outcomes, as we've seen in South and West Berkeley, through the CHIP.

361
00:54:34.180 --> 00:54:36.399
Board Room: investing in…

362
00:54:36.910 --> 00:54:47.530

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Board Room: Investing in… Thank you. Thank you. I'll send the rest of my comments. Thank you, I know, there's lots to talk about here, it's challenging. Do we have any other public comment?

363
00:54:49.910 --> 00:54:57.479
Board Room: Is there any public comment online for the community health improvement plan? There's one hand raised.

364
00:54:57.650 --> 00:55:03.830
Board Room: that, that's Matt. Matt, you should be able to… Unmute.

365
00:55:04.680 --> 00:55:10.589
Matt Matusiewicz: Super. Hi, everybody. My name's Matt. I, was part of the community steering committee that.

366
00:55:10.750 --> 00:55:24.949
Matt Matusiewicz: Was, on this trip. And I just wanted to point out just a couple of things that may be helpful for our council members, or those that are not as involved with public health to begin with. I know that a lot of

367
00:55:24.950 --> 00:55:37.880
Matt Matusiewicz: there was a lot of interest in talking about how to improve mortality from Alzheimer's disease, and there are a lot of things that are in this report, that I think push towards, having better

368
00:55:38.160 --> 00:55:57.750
Matt Matusiewicz: primordial, prevention. For example, diabetes is a high risk factor of stroke. Diabetes is causal of ischemic heart disease, diabetes is causal of hypertensive heart disease, like, all of these different things that are pushed at the very top of the City of Berkeley's, mortality,

369
00:55:57.800 --> 00:56:05.740
Matt Matusiewicz: And, you know, all of those things are associated or causal themselves of Alzheimer's disease, and other…

370
00:56:06.830 --> 00:56:20.680
Matt Matusiewicz: pieces of this are very, very interwoven within all of these things, and I can say, as somebody that works at the Department of Medicine at San Francisco General Hospital, that, if somebody is unable to

371
00:56:21.040 --> 00:56:32.659

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Matt Matusiewicz: have housing, their diabetes outcomes are going to be far, far worse, and so I think a lot of these are addressing what are called, like, primordial prevention levels.

372
00:56:32.780 --> 00:56:43.849
Matt Matusiewicz: So not only should the implementation of that look really, really strong at that, but also at the public health programming that is kind of working on,

373
00:56:44.430 --> 00:56:55.050
Matt Matusiewicz: you know, mitigating those risks. So, just kind of wanted to put that out there for folks that, aren't as familiar with the way that a lot of public health diseases sort of implement.

374
00:56:57.690 --> 00:56:58.640
Board Room: Thank you.

375
00:57:00.880 --> 00:57:07.130
Board Room: Okay. Alright, well, I will move us on to comments then. We have a comment from Councilmember Humbert.

376
00:57:08.030 --> 00:57:09.419
Board Room: Thank you, Madam Mayor.

377
00:57:10.260 --> 00:57:21.329
Board Room: I want to begin by expressing my immense gratitude to you, Director Gilman, to you, Deputy Director Bustamante, and other members of the HHCS team.

378
00:57:21.650 --> 00:57:25.870
Board Room: Janice… Janice Chin, or Janice Chin, Kelly Knox.

379
00:57:26.210 --> 00:57:39.840
Board Room: Patricia Zielcita, and Catherine Roseman. You know, I apologize, there are too many other people, to name, but I also want to thank our partners at JSI, and especially members of the Community Steering Committee.

380
00:57:40.560 --> 00:57:47.079
Board Room: The Berkeley Wellness Blueprint is a very candid document, and that's a great strength.

381
00:57:47.510 --> 00:57:54.570

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Board Room: The 13-year life expectancy gap between the Berkeley Hills, my part of the town, or part of, at least.

382
00:57:54.770 --> 00:57:59.609
Board Room: part of the Berkeley Hills, and South and West Berkeley is a stark and necessary finding.

383
00:57:59.790 --> 00:58:06.910
Board Room: that should anchor every health and equity conversation we have going forward. And I've heard other people say that in different words.

384
00:58:07.370 --> 00:58:25.820
Board Room: The four priority areas, housing, environmental health, behavioral health, and community safety, those make all the sense in the world to me. Housing being number one. Reflect what residents actually told us, and the goals of closing that gap and building genuine community partnership, I think, are the right ones.

385
00:58:25.940 --> 00:58:34.129
Board Room: I'm particularly glad that the plan emphasizes prevention and cross-sector collaboration, because health is made in neighborhoods.

386
00:58:34.290 --> 00:58:35.990
Board Room: Not just in cities.

387
00:58:36.180 --> 00:58:41.580
Board Room: But we've got to be clear-eyed about the constraints ahead. The budget environment is severe.

388
00:58:41.840 --> 00:58:51.519
Board Room: That's an understatement. Federal funding, that our most vulnerable residents depend on is under threat. That's also an understatement.

389
00:58:51.650 --> 00:58:55.369
Board Room: And the community organizations this plan relies on are absorbing

390
00:58:55.600 --> 00:58:59.589
Board Room: those same shocks. We'll not be able to act on everything.

391
00:58:59.870 --> 00:59:03.140

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Board Room: Or maybe even most things in this document in the near term.

392
00:59:03.380 --> 00:59:05.629
Board Room: And I think saying that we can

393
00:59:06.000 --> 00:59:22.130
Board Room: is potentially a disservice to the community members who invested their trust in this process. What we can, and I think we have to do, is use this framework actively where we have key decision points, such as in budget decisions, that's probably number one.

394
00:59:22.360 --> 00:59:27.020
Board Room: Departmental work plans, grant applications, equally important.

395
00:59:28.770 --> 00:59:43.260
Board Room: and perhaps more importantly, public safety and roadway safety, and I'm glad that that showed up in the report. And I think it's more than just walk… pedestrian safety and bike safety, it's public safety in general.

396
00:59:43.690 --> 00:59:48.900
Board Room: I'm a little uncertain how we're gonna build out the accountability framework in our… in the context of

397
00:59:49.050 --> 00:59:51.779
Board Room: Our limited, constrained resources.

398
00:59:52.000 --> 01:00:08.569
Board Room: But to the extent we actualize this plan, I think that needs to be a top priority. I'd ask Director Gilman and HHCS to come back to Council, or the City Manager with a concrete accountability framework, even a minimal one that reflects our current resource reality.

399
01:00:08.820 --> 01:00:23.619
Board Room: to the CSC members here today, I hope you will stay engaged and hold us to the maximum extent, hold us to this to the maximum extent feasible. So again, a huge thank you. This is a really important document.

400
01:00:23.790 --> 01:00:28.480
Board Room: I look forward to this plan being another guiding light in our decision.

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401
01:00:28.630 --> 01:00:33.430
Board Room: Decision-making as we work to overcome these health challenges and disparities that just…

402
01:00:33.550 --> 01:00:41.510
Board Room: should be, you know, are absolutely, a prime priority of mine. The disparities are just…

403
01:00:41.830 --> 01:00:48.069
Board Room: continue to be shocking. Every time I see them, I, you know, it makes me very sad. Thank you.

404
01:00:49.350 --> 01:00:51.710
Board Room: Thank you, Councilmember. Councilmember Trekup?

405
01:00:51.910 --> 01:01:02.860
Board Room: Thank you, Madam Mayor. Thank you again, Director Gilman and Deputy Director Bustamante, for your presentation, and…

406
01:01:03.030 --> 01:01:09.239
Board Room: to all who made this report possible, too many people to list, so I'm not going to.

407
01:01:10.490 --> 01:01:15.690
Board Room: I'm so glad that you emphasized the importance of the report being actionable.

408
01:01:16.080 --> 01:01:27.649
Board Room: I look forward to, along with my colleagues on the Council, to engaging with it more deeply in the coming months, and collaborating with you and your team

409
01:01:27.880 --> 01:01:29.189
Board Room: and my colleagues.

410
01:01:29.800 --> 01:01:35.689
Board Room: Particularly in my role as chair of the Health and Life Enrichment and Equity Committee.

411
01:01:36.590 --> 01:01:42.139
Board Room: to, look at, how existing policies

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412
01:01:42.560 --> 01:01:48.279
Board Room: Do or do not bring the report's recommendations into meaningful effect.

413
01:01:48.570 --> 01:01:51.430
Board Room: And where they do not, what is needed.

414
01:01:53.840 --> 01:02:04.170
Board Room: Within our community, through meaningful, policy, and legislative, direction.

415
01:02:04.630 --> 01:02:08.630
Board Room: I will note, also, Oh.

416
01:02:09.990 --> 01:02:11.740
Board Room: Was that it?

417
01:02:12.130 --> 01:02:23.620
Board Room: You're gifted some time from Councilmember Taplan. Oh, thank you. That was… that could not have possibly been 5 minutes. But I am almost complete.

418
01:02:24.440 --> 01:02:30.980
Board Room: I will note, and, I, I want to echo, Councilmember Humbert's comments,

419
01:02:31.220 --> 01:02:36.739
Board Room: Of course, one of the guiding principles is to balance ambition with feasibility.

420
01:02:36.880 --> 01:02:49.469
Board Room: And we have a tough situation on our hands. I will note, for instance, just around environmental factors, which was, or environmental solutions, such as tree planting.

421
01:02:49.560 --> 01:03:07.180
Board Room: I know that we have lost some needed, grant funding for that. How do we backfill, or are we able to? And so, within establishing a long-term cycle, the, the final guiding principle, laid out.

422
01:03:07.290 --> 01:03:17.640

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Board Room: Even though this is not intended to be an advocacy document, and I recognize it's not, to the extent that this is a requirement by the state, I think

423
01:03:17.750 --> 01:03:21.439
Board Room: We would be remiss not to also tell the state

424
01:03:21.620 --> 01:03:27.280
Board Room: This is what needs to be funded in order for us to effectuate these goals.

425
01:03:27.310 --> 01:03:44.909
Board Room: So I… I know we have, another, another agenda item tonight around legislative priorities, and, some of this may come up, but I… I really, given that, every jurisdiction and ours is no different.

426
01:03:44.910 --> 01:03:52.689
Board Room: Is trust into this very challenging environment, especially right now, where we have to do a lot more with a lot less.

427
01:03:53.160 --> 01:04:00.489
Board Room: I think we need to be able to use any… any and every lever of power and opportunity.

428
01:04:00.610 --> 01:04:02.900
Board Room: To tell, the state

429
01:04:03.180 --> 01:04:09.550
Board Room: what we need in order to pursue the goals of this plan. Thank you so much.

430
01:04:11.100 --> 01:04:13.939
Board Room: Thank you. Other comments from my colleagues?

431
01:04:14.070 --> 01:04:16.640
Board Room: Yes, Councilmember, or Vice Mayor Lunapara.

432
01:04:16.870 --> 01:04:23.680
Board Room: Thank you. I just wanted to thank staff and thank the community members who have been involved in this process. This was a great presentation. Thanks.

433

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01:04:25.230 --> 01:04:26.449
Board Room: Thank you. Anyone else?

434
01:04:27.260 --> 01:04:28.100
Board Room: Okay.

435
01:04:28.120 --> 01:04:37.720
Board Room: Oh, I have a couple other questions that came up for me. As we were talking about Alzheimer's and dementia, something that I was wondering about is…

436
01:04:37.720 --> 01:04:51.520
Board Room: is it perhaps a very high cause, partially because people are living longer, so they're more likely to have Alzheimer's and dementia? Because unless it's early onset, then typically folks would be later on in their years.

437
01:04:51.810 --> 01:04:53.290
Board Room: Can you speak to that at all?

438
01:04:53.800 --> 01:05:02.840
Board Room: I'm sorry, we can get back to you on that, but I would need to consult with our public health officer. Yeah, no, that makes sense, thank you.

439
01:05:02.840 --> 01:05:15.629
Board Room: And I think similarly, as I was looking at the geographic information, one of the things that I was thinking about is, like, yes, of course, those disparities are quite stark, and I think that that is something that's really concerning to all of us. Many of us have…

440
01:05:15.630 --> 01:05:23.929
Board Room: commented on it, and I also think that typically, you know, houses that are up in the hills cost more, you know, houses that, you know, are in the flats

441
01:05:23.930 --> 01:05:30.550
Board Room: Cost less, meaning most likely that people who have more funding would live up in the hills, they would therefore have access to

442
01:05:30.550 --> 01:05:46.939
Board Room: you know, better healthcare, you know, gym memberships, you know, better just care generally for themselves, healthier produce. And so, you know, of course there are structural things that… systemic things

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that we need to be looking at our city that might be impacting the geographic, health…

443
01:05:47.070 --> 01:06:00.270
Board Room: or age life expectancy disparities, and some of those things are going to be sort of embedded in just based on who can afford to live in these different places. So I just want us to think about that as we're thinking about those discrepancies.

444
01:06:00.270 --> 01:06:19.900
Board Room: Okay, and then… and then also, I know many of these challenges that we've had are challenges that we've had for many, many years, and are, you know, not unique, of course, to our city, but are all around the country and the world. And I'm wondering, are there strategies that we've deployed that have had an impact on these numbers, on improving them? You know, are there things that have…

445
01:06:19.900 --> 01:06:24.820
Board Room: Helped us kind of see a jump, or is this sort of like a steady progress situation?

446
01:06:25.950 --> 01:06:34.730
Board Room: I think the way that I would look at it is more, it's just, you know, some of these are lagging indicators that, you know, you might not see for a few years, you know, as we look at those.

447
01:06:34.910 --> 01:06:42.659
Board Room: I think… The thing that's new that we've seen better results on is breaking down the silos.

448
01:06:42.780 --> 01:06:45.630
Board Room: And, you know,

449
01:06:46.510 --> 01:06:59.419
Board Room: you know, one of the examples that I use is, we may at times, and I've seen this in my department, where we'll have 3 different city departments interacting with a family, mental health side, environmental health side.

450
01:06:59.870 --> 01:07:07.979
Board Room: And we're not, we're not coordinating care, so breaking down the silos as we try to move these interventions forward is a big part of what we need to do.

451

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01:07:08.040 --> 01:07:23.830
Board Room: It's also… it's also being better fiscal stewards. So when… the magic happens is when you can get primary care, mental health, behavioral health, the social net system, all working together, that's… that's where you can start to see the real movement.

452
01:07:24.880 --> 01:07:40.449
Board Room: I love that. I think that's really important, because it's really about being more efficient with the resources and the systems and the people we have in place already, and I think that that's important across all levels of our government, so I'm really glad to hear about breaking down those silos, and

453
01:07:40.450 --> 01:07:50.610
Board Room: And especially given, as everyone has commented on already, but just our concerns about our budget deficit, $32 million is really significant, and,

454
01:07:50.610 --> 01:07:57.720
Board Room: you know, I don't want us to… I think Paul has said this to me before, our city manager, that you can't… you shouldn't be doing,

455
01:07:57.720 --> 01:08:17.849
Board Room: more with less. It's kind of really, like, we need to be adjusting and right-sizing and making sure that we're being realistic so that we also don't have burnout, because I'm particularly concerned about the people who do the work in our communities and serve these families, and see, like, year after year, you know, I want them to feel like they're seeing results, and…

456
01:08:17.850 --> 01:08:23.770
Board Room: That we're actually able to, to serve people and, and help them get better health outcomes, so…

457
01:08:23.800 --> 01:08:33.570
Board Room: I think when I see this information, you know, what I worry about is just not having enough resources and not being able to, like, kind of wrap our arms around

458
01:08:33.620 --> 01:08:51.580
Board Room: these massive problems, because they are really big problems. So I appreciate knowing also that there's kind of a process moving forward that you all have to continue to engage the community and figure out, like, what are these areas that we can be, better collaborating, connecting with the community, and breaking down those silos.

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459
01:08:51.689 --> 01:09:01.510
Board Room: And that was one of the other comments I was gonna make, highlighting the insight that more transparency and collaboration are needed to increase trust and effectiveness of health improvement efforts.

460
01:09:01.560 --> 01:09:14.339
Board Room: Transparency, building trust, community engagement are all really important to me, and I think that, again, is something important throughout our city. It's something we've been trying to do on council, even with each other, just, you know, connecting with each other and building relationships.

461
01:09:14.410 --> 01:09:21.019
Board Room: And the focus on housing, environmental health, behavioral health, community safety make a lot of sense.

462
01:09:21.120 --> 01:09:24.530
Board Room: And housing in particular stands out to me.

463
01:09:25.040 --> 01:09:37.359
Board Room: when I saw last year's community health assessment that approximately 60% of African Americans and 60% of Latinx households in Berkeley pay more than 30% of their income for rent.

464
01:09:37.399 --> 01:09:48.980
Board Room: It's really clear that addressing housing instability and affordability remains a high priority, and that is a huge priority for me, and I know my council, many of my council members as well.

465
01:09:49.319 --> 01:10:02.320
Board Room: stable, safe housing that's free of environmental risks, like mold, pollution, have a direct impact on health outcomes. I… I really also am concerned about people not understanding the… their legal,

466
01:10:02.510 --> 01:10:03.510
Board Room: rights.

467
01:10:03.660 --> 01:10:25.670
Board Room: Around this in particular. I think so many people that I speak with, especially young people, are afraid to call out when there are these concerns about living in an unsafe housing situation, and so I do also want us to keep in mind making sure people understand their rights and their ability to get health inspections, you know, inspections on their apartments.

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468
01:10:25.670 --> 01:10:27.990
Board Room: If they have things like mold,

469
01:10:28.170 --> 01:10:38.519
Board Room: I've even talked to my own staff about that, advocating for themselves in that way. And then I'm really glad to see the plan focus on prevention, especially when it comes to mental health.

470
01:10:38.760 --> 01:10:47.120
Board Room: And I'm also really concerned about the shift in mental health and behavioral health funding away from prevention and early intervention.

471
01:10:48.420 --> 01:10:59.799
Board Room: it's just… it is concerning to me that some of these shifts that we're seeing, especially from the federal government, I don't want that to impact how we're doing things here, and that's really essential, so…

472
01:10:59.830 --> 01:11:12.900
Board Room: Just some thoughts that I have on your report, and just really, again, thank you all so much for taking the time to come and present this to us, and also to keep us updated, because I think that's really important. I think this really helps ground us in, you know.

473
01:11:12.900 --> 01:11:20.860
Board Room: really what's happening with people's everyday lives, and how our different policies are potentially impacting their wellness and stability, so…

474
01:11:20.860 --> 01:11:22.009
Board Room: Thank you all.

475
01:11:22.680 --> 01:11:26.489
Board Room: I believe that is our final…

476
01:11:27.070 --> 01:11:30.219
Board Room: Item on here, and we,

477
01:11:32.140 --> 01:11:40.790

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Board Room: And we don't need to do anything to receive it, but I do want to make sure that we acknowledge, the steering committee, so I'll pass it back over to you all.

478
01:11:41.940 --> 01:12:01.739
Board Room: Thank you, Mayor and Council. So at this, time, we wanted to, recognize and appreciate the 10 members of our Community Steering Committee who were so critical to, the CHA and the CHIP development process. And I'm going to read off names, but essentially the certificate,

479
01:12:01.740 --> 01:12:12.430
Board Room: is appreciating these members for their dedication in creating the Community Health Assessment and the Improvement Plan, and for their service in creating healthier Berkeley communities.

480
01:12:12.860 --> 01:12:20.900
Board Room: And as Tanya reads off names of individual members, if you wouldn't mind stepping up to the front, and then we'll be ready to take a picture.

481
01:12:25.270 --> 01:12:28.840
Board Room: So, I'd like to call up Makai Freeman.

482
01:12:29.260 --> 01:12:33.990
Board Room: If they're here… Michael Rodriguez.

483
01:12:36.380 --> 01:12:37.910
Board Room: Caitlin Cole?

484
01:12:43.210 --> 01:12:44.940
Board Room: Penelope Collins.

485
01:12:48.730 --> 01:12:52.699
Board Room: Matt Ann, I know he's not here, because he spoke earlier via Zoom.

486
01:12:53.430 --> 01:12:54.450
Board Room: Thank you, Matt.

487
01:12:55.630 --> 01:12:57.130
Board Room: Clariza Rios?

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488
01:12:59.690 --> 01:13:02.280
Board Room: Israel Nicodemos?

489
01:13:06.610 --> 01:13:09.080
Board Room: Rosillo Almaguer Andrade.

490
01:13:13.020 --> 01:13:14.750
Board Room: Isabella Ledesma?

491
01:13:17.280 --> 01:13:19.019
Board Room: and J.W. Fry.

492
01:13:21.440 --> 01:13:22.880
Board Room: Thank you, everyone.

493
01:13:29.030 --> 01:13:38.399
Board Room: We gotta get more people in.

494
01:13:39.170 --> 01:13:41.260
Board Room: We have JSI here.

495
01:14:24.270 --> 01:14:26.380
Board Room: Thank you.

496
01:14:47.650 --> 01:14:52.329
Board Room: Okay, thank you everyone, and with that, is there a motion to adjourn?

497
01:14:52.770 --> 01:14:55.210
Board Room: Some of… Second.

498
01:14:55.370 --> 01:15:03.329
Board Room: Second. There we go, second. Alright, and as long as there's no opposition, I will have us all marked as ayes, and we will be adjourned.

499
01:15:03.780 --> 01:15:06.670
Board Room: Alright, meeting adjourned. Thanks, everyone.

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